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Name: لمياء محمد احمد اسماعيل National ID: 28310011335424	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

لمياء
محمد احمد اسماعيل كبيش
١٢ ش يعقوب القبطي
مصر القديمة - القاهرة

١٩٨٣/١

٢٨٣١٠٠١١٣٣٥٤٢٤

IF8653999



MDT FORM

Name:	لمياء محمد احمد اسماعيل	MRN:	11122
Age:	43 Y	ID:	28310011335424
Menopausal Sta	PRE	Phone:*	01000762868
NRS SCORE:	PS 0	Marital Status:	MARRIED
BMI:	25	Family History:	-VE
Gender		Reason Of MDT:	NEW CASE
CO Morbidity:	NAD	ECOG Performance Status	☒ 0 ☐ 1 ☐ 2 ☐ 3 ☐ 4
Surgical History:*	CS		

Clinical Data:

BX: IDC WITH LOBULAR DIFFERENTIATION

ER: 8/8 **PR:** 8/8 **HER2:** 0+ **KI67:** 15

History:

Sonomammography:

ACR C
RT MASS AND NON MASS LESIONS OVER ALL 10/3/3 CM
ENLARGED AX LNS AT LEVEL 1

Biomarker/Molecular

Mets Workup:

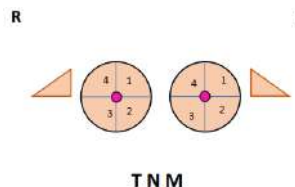
FREE

Recommendations:

FOR CLIP BRACKETING THEN NACT THEN RT WLE +SLNB

MDT Outcome & Plan

ECHO / PORTA CATH
AC X 4C / TXT X 4C
OFS WITH ZOLADEX
CLIP BRACKETING THEN NACT THEN RT WLE +SLNB



Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د ندي حنين	د محمد نيراي		

Patient Name	لمياء محمد احمد	Case No.	339238
Sex/Age	Female 42 Y	Patient No.	24271
Registered Date	29/6/2026 12:57:30 PM	Referred By Dr.	Interventional Radiology
Validation Date	20/7/2026 02:13:25 PM	Branch	Gustave Roussy
Reporting Date	21/7/2026 08:49:40 AM	Pathology No.	2043/26

Clinical Data: Right breast 5 O'clock area of parenchymal distortion (BIRADs 4b).

Nature of Specimen: Tru-cut biopsy, collected on 29/06/2026, at 1:00 pm. Cold ischemia time is 20 seconds.

Gross Examination:

- Multiple fragmented grey, white tissue cores, ranging in size between 0.2 cm & 1.0 cm long, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- **Histologic type:** Invasive duct carcinoma with lobular differentiation.
- **Histologic grade:** 2 (differentiation 3, pleomorphism 2, mitosis 1).
- **Ductal carcinoma in situ:** Minimal low grade ductal carcinoma in situ (Solid pattern).
- **Lobular cancerization:** Present.
- **Microcalcification:** Present
- **Angioinvasion:** Present
- **Perineural invasion:** Absent
- **Tumor infiltrating lymphocytes:** Mild.

Results:

* IPT (All controls showed appropriate reactivity):



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- ER: Strong nuclear reaction in 98% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: Faint incomplete membrane reaction in less than 10% of tumor cells (score 0+).
- Ki67%: 15%.

Conclusion:

Right breast mass, Tru-cut biopsy:

- Invasive duct carcinoma with lobular differentiation, Grade 2.
- Minimal low grade ductal carcinoma in situ (Solid pattern).

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Ultra-low (score 0+).
- Ki67%: 15%.

**Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%

and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

** Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of



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the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+ based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature

Dr. Walaa Ibrahim Ghonaim.



Patient's Name: Lamia Mohammed Ahmed Ismail
Date: 2026-08-05
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST,
ABDOMEN AND PELVIS REVEALED:**

Known case of breast CA.

I- CHEST:

- Suspected right breast lesion measuring 34 x 21 mm with multiple right axillary lymph nodes largest measuring 34 x 15 mm for sono-mammography.
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Multiple subcentimetric para-aortic and bilateral external iliac lymph nodes.
- No ascites.

Opinion:

Patient's Name: Lamia Mohammed Ahmed Ismail
Date: 2026-08-05
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

- ❖ Suspected right breast lesion with right axillary lymph nodes for sono-mammography.
- ❖ No pulmonary or hepatic metastatic lesions.

Dr.sherif Wagdy

اسم المريض بالإنجليزية: Lamiaa Mohamed Ahmed	كود المريض: 97792
تاريخ التقرير: 2026/08/03 11:06:12 AM	السن: ٤٢ سنة ١٠ شهر

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: 43 years old female patient with pathologically proven invasive ductal with lobular differentiation carcinoma grade II (+ve ER, +ve PR, -ve HER2 and Ki67= 15 %)

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: Moderate background enhancement.
ACR: C (heterogenous dense)

Right breast:

- **Low energy (Non contrast) images:** Right breast shows middle 1/3 focal asymmetry with architectural distortion. Regional amorphous microcalcifications are seen coinciding measuring about 7x5.5x3 cm, about 2 cm from the nipple
- **Recombined (Post contrast) images:** right breast shows malignant enhancing process inform of mass & non mass areas anterior & posterior to the mass enhancement
 - mass enhancement of non homogeneous pattern, moderate conspicuity, middle 1/3 irregular of indistinct margins measures 4.5X2.9X3.5 cm
 - Anterior non mass non homogeneous enhancement, mild conspicuity and measuring about 2.9x1.5 cm
 - posterior non mass non homogeneous enhancement, mild conspicuity and measuring about 2x1.5 cm
 - overall non mass & mass enhancement, measuring about 10x 3 x 3 cm , from nipple 2 cm , larger than the regional microcalcifications.
- **Complementary US:**
 - The right breast shows irregular shaped spiculated hypoechoic lesions located at 5 o'clock and measures about 3.3x 2.7 cm.
 - With non mass lesion extended anterior measured about 1.8x1.5 .
 - With non mass lesion extended posterior measured about 1.5x1 cm .
 - Few enlarged axillary lymph node at level I with focal cortical thickening, largest one measuring about 28 x 13 mm with focal cortical thickening measuring about 9 mm.

Left breast:

- **Low energy (Non contrast) images** No asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images** no mass or non mass enhancement.
- Normal skin / nipple / areolar complex.
- **Complementary ultrasound:**
 - Left breast shows few simple cysts .
 - No enlarged axillary LNs.

IMPRESSION:

- **RIGHT BREAST:** (BIRADS 6= pathological proven) mass and non-mass enhancement lesions as described above.
- **LEFT BREAST:** (BIRADS 2= normal) mild fibrocystic changes

Best regards,

Dr. lamiaa adel, MD

Dr marwa hakim, Msc

Name:	Lmiaa Mohamed Ahmed Ismail	Age :	43
Referred by:	Prof. Dr/ ...1	Date :	13/06/2026
Examination:	MAMMOGRAPHY		

BILATERAL SONO-MAMMOGRAPHY:

- Both breasts show normal size, shape, nipple, areola, skin and subcutaneous tissue thickness.
- Breast parenchyma is extremely dense, decreasing the sensitivity of the mammo-gram (ACR-D).

- Evidence of cluster of micro calcifications at lower inner quadrant .

Complimentary sonography revealed:

- **The right breast show** two ill-defined hypo echoic lesions showing irregular outline one seen at 5 o'clock measures 19x10 mm , another one seen at 6 o'clock measures 12x15mm (about 15mm apart from the previous lesion) .
- Normal caliber of lactiferous ducts with no ectatic changes.
- **The right axilla show** multiple reactive lymph nodes , yet there is three lymph nodes show mild suspicious criteria as follow :
 - ↘ - Two show loss of central hilum with preserved oval shape measures 20x10mm & 11x7mm respectively .
 - One show focal cortical thickness measures 9mm in thickness , it measures 28x13mm .
- No suspicious left axillary lymph nodes

CONCLUSION:

- Right breast two suspicious lesions (at same quadrant) BIRADS IVb.....for FNAC .
- Mild suspicious right axillary lymph nodes .

Best regards,
Dr. Heba Mohamed